Statement of Deficiency Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

Comprehensive Life Resources - Pearl Youth Residence
1201 South Proctor Street
Tacoma, WA 98405-2047
Agency Name and Address

Melissa Olson
Administrator

Investigation	09/18/20-10/20/20	JAMC03
Inspection Type	Investigation Dates	Investigator Number
2020-13309	BHA.FS.61067737	Mental Health Services
Case Number	License Number	BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
WAC 246-341-1126(1)(b) Mental health inpatient services – Policies and procedures – Adult. In addition to meeting the agency licensure, certification, administration, personnel, and clinical requirements, in WAC 246-341-0100 through 246-341-0650, and the applicable inpatient service requirements I WAC 246-341-18 through 246-341-1132, an inpatient facility must implement all of the following administrative requirements: (1) Policies to ensure that services are provided in a secure environment. "Secure" means having; (b) Visual monitoring, either by line of sight or camera as appropriate to the individual.	Based on record review, policy and procedure review, and interview, the facility failed to implement policies and procedures that ensure line of sight monitoring as appropriate to the individual for 1 of 1 Residents reviewed (Resident #1). Failure to implement policies and procedures that ensure line of sight monitoring as appropriate to the individual may result in patient harm. Findings included: 1. Document review of facility policy titled "Psychological, Psychosocial, and Behavior Management – Personal / Room Inspection and Line-of-sight", dated	

	4/2018, showed that the facility had a policy for monitoring residents for safety. "A resident who poses a danger to him / herself and is placed on-line of sight and must be able to be seen by staff at all times..." The policy showed that residents that are on line-of-sight are not to be alone with a staff in a room. The policy showed that 15-minute checks are to be documented in the clinical record for residents that are assigned to line-of-sight monitoring. 2. Review of Resident #1's clinical record showed that the facility failed to document completed 15-minute checks, based on the following: a. Review of the document titled "Resident Observation Record", dated 08/17/20 showed that Resident #1 was on 15-minute intervals for observation checks. There are no staff initials for observation checks completed from 8:00 PM to 9:45 PM on 8/17/20. b. Review of the document titled "Resident Observation Record", dated 08/20/20 showed that Resident #1 was on 15-minute intervals for observation checks. There is no 15-minute check documented for noon. c. Review of the document titled "Resident Observation Record", dated 09/17/20 showed that Resident #1 was on 15-minute intervals for observation checks. There are no staff initials for observation checks completed from 11:00 PM to 11:45 PM on 09/17/20. d. Review of the document titled "Resident Observation Record", dated 09/24/20 showed that	

	Resident #1 was on 15-minute intervals for observation checks. There are no staff initials for the observation check completed at 5:45 AM on 09/24/20. e. Review of the document titled "Resident Observation Record", dated 09/29/20 showed that Resident #1 was on 15-minute intervals for observation checks. There are no staff initials for the observation check completed at 5:45 PM on 09/29/20. 3. Review of Resident #1's clinical record, and interview showed that Resident #1 was on line-of-sight monitoring and the facility failed to maintain line-of-sight monitoring, based on the following: a. During an interview on 10/02/20 at 10:15 AM, Staff A, Administrator, stated that Resident #1 was assigned to line-of-sight, with authorization to increase to 1:1 monitoring (staff within arms-reach of Resident, monitoring resident at all times) as needed. Staff A stated that Resident #1 swallowed an AA battery on 09/23/20 during a session with a mental health counselor. "There was a family session, the family session went okay on Zoom, the Therapist turned away and [Resident #1] took the remote out he the drawer and took out he batteries... [Resident #1] was able to swallow a battery". b. Review of an incident report dated 09/23/20 showed that Resident #1 swallowed the AA battery on 09/23/20. "[Resident #1] requested a PRN reporting that she was feeling anxious. [Staff G, Therapist] opened the office door to request RC (Residential Counselor) staff to alert nursing of the	

	need for PRN. When [staff G] turned back around [Resident #1] had grabbed the remote control to the office TV and had taken an AA battery out".	

Introduction

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring

Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification, or have questions about deficiencies you must contact the investigator who conducted the onsite investigation.

Washington State Department of Health
PLAN OF CORRECTION

Investigation Dates: 9/18/20-10/20/20 Investigator Number: JAMC03 License Number: BHA.FS.61067737 Case Number: 2020-13309	Comprehensive Life Resources Pearl Youth Residence (PYR) 1201 South Proctor Street Tacoma, WA 98405-2047

Deficiency Number and Rule Reference	Correction	Responsible Party	Implementation Date	Maintaining Correction
WAC 246-341-1126(1)(b) Mental health inpatient services – Policies and procedures – Adult. In addition to meeting the agency licensure, certification, administration, personnel, and clinical requirements, in WAC 246-341-0100 through 246-341-0650, and the applicable inpatient service requirements I WAC 246-341-18 through 246-341-1132, an inpatient facility must implement all of the following administrative requirements: (1) Policies to ensure that services are provided in a secure	1. PYR Policy, PSY-03, Psychological, Psychosocial, and Behavior Management – Personal / Room Inspection and Line-of-sight will be reviewed. 2. After PYR Policy, PSY-03 has been reviewed and revised (if needed), it will be shared with staff. 3. The Observation Log (paper form) will be audited for initials. Incomplete logs will be returned to Shift Supervisors. 4.The Observation Log (currently being completed on paper and scanned in) will be made into an electronic record. This version will be created to make the initials section required. 5.The electronic log will be implemented within two weeks after PYR receives the devices for milieu use (they have been ordered already).	1.Program Director will ensure PSY-03 is reviewed 2.Shared 3. Overnight Shift 4. Administrative Services Manager 5. Administrative Services Manager	1. Reviewed (and updated if necessary) 1/4/21 2.Shared with staff by 1/8/21 3. 12/4/20 4. 1/5/21 5. Two weeks after the devices are received.	1.& 2. Administrative Services Manager will check that the policy has been reviewed and shared with staff on or before 1/8/2021 3. Administrative Services Manager will collect the logs and return those that are incomplete for correction. 4. Quality Improvement Associate will check in every two weeks with the Administrative Services Manager to ensure the Observation Log is being used and completed (starting 1/12/21) and then

environment. "Secure" means having; (b) Visual monitoring, either by line of sight or camera as appropriate to the individual.	6. Pearl Youth Residence has amended the Milieu Expectations (Exhibit A) to include a Battery/ Shoelace/String/Metal free milieu. The residents will not have access to batteries unless they are checked out. The remote control batteries will be accessible only with a screwdriver. They can check out calculators. 7. Training to remind staff of safety expectations	6. Milieu Manager 7. Milieu Manager	6. 11/12/20 7.11/13/20	determine if the log is working effectively. 6. Three random checks to ensure that all batteries are locked up to be completed by Milieu Manager by 11/13/20 12/15/20 12/30/20 And reported to Program Director

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

November 24, 2020

Comprehensive Life Resources – Pearl Youth Residence
1201 South Proctor Street
Tacoma, WA 98405-2047

Re: Case Number: 2020-13309 and 2020-13310
License Number: BHA.FS.61067737 and RTF.FS.61067805
Acceptable Plan of Correction
Date(s) of Investigation: 09/18/20 – 10/20/20

Dear Melissa Olson:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your facility, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected.

The Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

Investigator: JAMC03
Department of Health
HSQA/Office of Health Systems Oversight
PO Box 47874
Olympia, Washington 98504-7874